Patient Name	: Miss.SMITA SHARMA	Collected	: 08/Aug/2025 10:12AM
Age/Gender	: 27 Y 11 M 24 D/F	Received	: 08/Aug/2025 11:41AM
UHID/MR No	: SCHI.0000031585	Reported	: 08/Aug/2025 01:09PM
Visit ID	: SCHIOPV51678	Status	: Final Report
Ref Doctor	: Dr.SELF	Sponsor Name	: CHECKMED PRIVATE LIMITED
Emp/Auth/TPA ID	: 1015234		

DEPARTMENT OF HAEMATOLOGY

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Test Name	Result	Unit	Bio. Ref. Interval	Method
COMPLETE BLOOD COUNT (CBC) , *WHOLE BLOOD EDTA*				
HAEMOGLOBIN	**11.8**	g/dL	12-15	Spectrophotometer
PCV	**35.20**	%	36-46	Electronic pulse & Calculation
RBC COUNT	3.9	Million/cu.mm	3.8-4.8	Electrical Impedence
MCV	90.1	fL	83-101	Calculated
MCH	30.3	pg	27-32	Calculated
MCHC	33.7	g/dL	31.5-34.5	Calculated
R.D.W	13	%	11.6-14	Calculated
TOTAL LEUCOCYTE COUNT (TLC)	6,550	cells/cu.mm	4000-10000	Electrical Impedance
DIFFERENTIAL LEUCOCYTIC COUNT (DLC)				
NEUTROPHILS	72.3	%	40-80	Flow cytometry
LYMPHOCYTES	21.5	%	20-40	Flow cytometry
EOSINOPHILS	**0.7**	%	1-6	Flow cytometry
MONOCYTES	5.4	%	2-10	Flow cytometry
BASOPHILS	0.1	%	0-2	Flow cytometry
CORRECTED TLC	6,550	Cells/cu.mm		Calculated
ABSOLUTE LEUCOCYTE COUNT				
NEUTROPHILS	4735.65	Cells/cu.mm	2000-7000	Calculated
LYMPHOCYTES	1408.25	Cells/cu.mm	1000-3000	Calculated
EOSINOPHILS	45.85	Cells/cu.mm	20-500	Calculated
MONOCYTES	353.7	Cells/cu.mm	200-1000	Calculated
BASOPHILS	6.55	Cells/cu.mm	0-100	Calculated
Neutrophil lymphocyte ratio (NLR)	3.36		0.78- 3.53	Calculated
PLATELET COUNT	263000	cells/cu.mm	150000-410000	Electrical impedence
MPV	**7.2**	Fl	8.1-13.9	Calculated

Dr.Yachika
M.B.B.S,M.D(Pathology)
consultant pathologist

SIN No:BED250069885

Patient Name	: Miss.SMITA SHARMA	Collected	: 08/Aug/2025 10:12AM
Age/Gender	: 27 Y 11 M 24 D/F	Received	: 08/Aug/2025 11:42AM
UHID/MR No	: SCHI.0000031585	Reported	: 08/Aug/2025 01:12PM
Visit ID	: SCHIOPV51678	Status	: Final Report
Ref Doctor	: Dr.SELF	Sponsor Name	: CHECKMED PRIVATE LIMITED
Emp/Auth/TPA ID	: 1015234		

DEPARTMENT OF BIOCHEMISTRY

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Test Name	Result	Unit	Bio. Ref. Interval	Method
GLUCOSE, FASTING , *NAF PLASMA*	86	mg/dL	70-100	GOD - POD

Comment:

As per American Diabetes Guidelines, 2023

Fasting Glucose Values in mg/dL	Interpretation
70-100 mg/dL	Normal
100-125 mg/dL	Prediabetes
≥126 mg/dL	Diabetes
<70 mg/dL	Hypoglycemia

Note:

1.The diagnosis of Diabetes requires a fasting plasma glucose of > or = 126 mg/dL and/or a random / 2 hr post glucose value of > or = 200 mg/dL on at least 2 occasions.

2. Very high glucose levels (>450 mg/dL in adults) may result in Diabetic Ketoacidosis & is considered critical.

Dr.Yachika
M.B.B.S,M.D(Pathology)
consultant pathologist

SIN No:PLF02234739

Patient Name	: Miss.SMITA SHARMA	Collected	: 08/Aug/2025 10:12AM
Age/Gender	: 27 Y 11 M 24 D/F	Received	: 08/Aug/2025 03:43PM
UHID/MR No	: SCHI.0000031585	Reported	: 08/Aug/2025 04:12PM
Visit ID	: SCHIOPV51678	Status	: Final Report
Ref Doctor	: Dr.SELF	Sponsor Name	: CHECKMED PRIVATE LIMITED
Emp/Auth/TPA ID	: 1015234		

DEPARTMENT OF BIOCHEMISTRY

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Test Name	Result	Unit	Bio. Ref. Interval	Method
HBA1C (GLYCATED HEMOGLOBIN) , *WHOLE BLOOD EDTA*				
HBA1C, GLYCATED HEMOGLOBIN	5.4	%		HPLC
ESTIMATED AVERAGE GLUCOSE (eAG)	108	mg/dL		Calculated

Comment:

Reference Range as per American Diabetes Association (ADA) 2023 Guidelines:

REFERENCE GROUP	HBA1C %
NON DIABETIC	<5.7
PREDIABETES	5.7 – 6.4
DIABETES	≥ 6.5
DIABETICS	
EXCELLENT CONTROL	6 – 7
FAIR TO GOOD CONTROL	7 – 8
UNSATISFACTORY CONTROL	8 – 10
POOR CONTROL	>10

Note: Dietary preparation or fasting is not required.

1. HbA1C is recommended by American Diabetes Association for Diagnosing Diabetes and monitoring Glycemic Control by American Diabetes Association guidelines 2023.
2. Trends in HbA1C values is a better indicator of Glycemic control than a single test.
3. Low HbA1C in Non-Diabetic patients are associated with Anemia (Iron Deficiency/Hemolytic), Liver Disorders, Chronic Kidney Disease. Clinical Correlation is advised in interpretation of low Values.
4. Falsely low HbA1c (below 4%) may be observed in patients with clinical conditions that shorten erythrocyte life span or decrease mean erythrocyte age. HbA1c may not accurately reflect glycemic control when clinical conditions that affect erythrocyte survival are present.
5. In cases of Interference of Hemoglobin variants in HbA1C, alternative methods (Fructosamine) estimation is recommended for Glycemic Control
   A: HbF >25%
   B: Homozygous Hemoglobinopathy.
   (Hb Electrophoresis is recommended method for detection of Hemoglobinopathy)

Dr.Tanish Mandal
M.B.B.S,M.D(Pathology)
Consultant Pathologist

SIN No:EDT250015834

Patient Name	: Miss.SMITA SHARMA	Collected	: 08/Aug/2025 10:12AM
Age/Gender	: 27 Y 11 M 24 D/F	Received	: 08/Aug/2025 12:31PM
UHID/MR No	: SCHI.0000031585	Reported	: 08/Aug/2025 01:11PM
Visit ID	: SCHIOPV51678	Status	: Final Report
Ref Doctor	: Dr.SELF	Sponsor Name	: CHECKMED PRIVATE LIMITED
Emp/Auth/TPA ID	: 1015234		

DEPARTMENT OF BIOCHEMISTRY

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Test Name	Result	Unit	Bio. Ref. Interval	Method
LIPID PROFILE , *SERUM*				
TOTAL CHOLESTEROL	175	mg/dL	<200	CHE/CHO/POD
TRIGLYCERIDES	74	mg/dL	<150	Enzymatic
HDL CHOLESTEROL	49	mg/dL	>40	CHE/CHO/POD
NON-HDL CHOLESTEROL	126	mg/dL	<130	Calculated
LDL CHOLESTEROL	**111.2**	mg/dL	<100	Calculated
VLDL CHOLESTEROL	14.8	mg/dL	<30	Calculated
CHOL / HDL RATIO	3.57		0-4.97	Calculated
ATHEROGENIC INDEX (AIP)	< 0.01		<0.11	Calculated

Comment:

Reference Interval as per National Cholesterol Education Program (NCEP) Adult Treatment Panel III Report.
Below Table as per Lipid Association of India (LAI) (2023) and Cardiological Society of India (CSI) (2024) Guidelines and Consensus Statements for Dyslipidemia Management:

	Low Risk	Moderate Risk	High Risk	Very High Risk	Extremely High Risk
Total Cholesterol	< 200	< 200	200 – 239	≥ 240	≥ 240
Triglycerides	<150	< 150	150 – 199	200 – 499	≥ 500
LDL	< 100	100 – 129	130 – 159 Target for High Risk: < 70	160 – 189 Target for Very High Risk: < 50	≥ 190 Target for Extreme Risk – Category A,B :< 30,Category C: 10 – 15
HDL	≥ 60	≥ 60	M: <40, F: <50	<30	<30
Non-HDL Cholesterol	< 130	130 – 159	160-189 Target for High Risk: < 100	190 – 219 Target for Very High Risk: < 80	≥220 Target for Extreme Risk – Category A,B :< 60, Category C: 40 – 45

Note: **Low risk** – No known risk factor of cardiovascular disease. **Moderate risk** – Any one risk factor, eg; smoking/hypertension/diabetes mellitus etc. **High risk** – Two or more risk factors without any disease manifestation, chronic kidney disease, long-standing diabetes mellitus existing for >10 years, history of heterozygous familial hypercholesterolemia.
Very high risk – clinical evidence of coronary artery disease, long-standing diabetes mellitus existing for >20 years, etc. **Extremely high risk** – recurrent vascular events.
(Category A- CAD with >1 feature of High risk group, Category B- CAD with >1 feature of very high risk group or poly-vascular disease, Category C- Recurrent CAD with other significant risk factors as high lipoprotein a, extensive coronary calcium)

1. Measurements for Lipids (Especially Triglycerides) can show physiological (Dependent on diet,10-12 hrs fasting pre-test condition) & analytical variations.
2. Lipid Association of India (LAI) recommends screening of all adults (>20 yrs) for Atherosclerotic Cardiovascular Disease (ASCVD) risk factors with lipid profile testing. The association recommends testing also to include Apolipoprotein B & Lipoprotein (a) for stratification and defining LDL – C targets.

Dr.Yachika
M.B.B.S,M.D(Pathology)
consultant pathologist

SIN No:SR04936357

Patient Name	: Miss.SMITA SHARMA	Collected	: 08/Aug/2025 10:12AM
Age/Gender	: 27 Y 11 M 24 D/F	Received	: 08/Aug/2025 12:31PM
UHID/MR No	: SCHI.0000031585	Reported	: 08/Aug/2025 01:11PM
Visit ID	: SCHIOPV51678	Status	: Final Report
Ref Doctor	: Dr.SELF	Sponsor Name	: CHECKMED PRIVATE LIMITED
Emp/Auth/TPA ID	: 1015234		

DEPARTMENT OF BIOCHEMISTRY

CHECKMED - IIFB - AHC - PAN INDIA - FY2526

Test Name	Result	Unit	Bio. Ref. Interval	Method
LIVER FUNCTION TEST (LFT) , *SERUM*				
BILIRUBIN, TOTAL	0.30	mg/dL	0-1.2	DIAZO METHOD
BILIRUBIN CONJUGATED (DIRECT)	0.10	mg/dL	0.0-0.3	Calculated
BILIRUBIN (INDIRECT)	0.20	mg/dL	0-1.1	Dual Wavelength
ALANINE AMINOTRANSFERASE (ALT/SGPT)	14	U/L	<35	Visible with P-5-P
ASPARTATE AMINOTRANSFERASE (AST/SGOT)	20.0	U/L	14-36	UV with P-5-P
AST (SGOT) / ALT (SGPT) RATIO (DE RITIS)	**1.4**		<1.15	Calculated
ALKALINE PHOSPHATASE	73.00	U/L	38-126	p-nitrophenyl phosphate
PROTEIN, TOTAL	6.90	g/dL	6.4-8.3	Biuret
ALBUMIN	3.70	g/dL	3.5 - 5	Bromocresol Green
GLOBULIN	3.20	g/dL	2.0-3.5	Calculated
A/G RATIO	1.16		0.9-2.0	Calculated

Comment:

LFT results reflect different aspects of the health of the liver, i.e., hepatocyte integrity (AST & ALT), synthesis and secretion of bile (Bilirubin, ALP), cholestasis (ALP, GGT), protein synthesis (Albumin) Common patterns seen:

1. Hepatocellular Injury: *AST – Elevated levels can be seen. However, it is not specific to liver and can be raised in cardiac and skeletal injuries.*ALT – Elevated levels indicate hepatocellular damage. It is considered to be most specific lab test for hepatocellular injury. Values also correlate well with increasing BMI. Disproportionate increase in AST, ALT compared with ALP. AST: ALT (ratio) – In case of hepatocellular injury AST: ALT > 1In Alcoholic Liver Disease AST: ALT usually >2. This ratio is also seen to be increased in NAFLD, Wilsons's diseases, Cirrhosis, but the increase is usually not >2.Note- If both SGPT and SGOT are within reference range then AST:ALT (De Ritis ratio) does not have any clinical significance.
2. Cholestatic Pattern:*ALP – Disproportionate increase in ALP compared with AST, ALT. ALP elevation also seen in pregnancy, impacted by age and sex.*Bilirubin (Direct) and GGT elevated- helps to establish hepatic origin.
3. Synthetic function impairment:*Albumin- Liver disease reduces albumin levels, Correlation with PT (Prothrombin Time) helps.
4. Associated tests for assessment of liver fibrosis - Fibrosis-4 and APRI Index.

Dr.Yachika
M.B.B.S,M.D(Pathology)
consultant pathologist

SIN No:SR04936357

Patient Name	: Miss.SMITA SHARMA	Collected	: 08/Aug/2025 10:12AM
Age/Gender	: 27 Y 11 M 24 D/F	Received	: 08/Aug/2025 12:31PM
UHID/MR No	: SCHI.0000031585	Reported	: 08/Aug/2025 03:27PM
Visit ID	: SCHIOPV51678	Status	: Final Report
Ref Doctor	: Dr.SELF	Sponsor Name	: CHECKMED PRIVATE LIMITED
Emp/Auth/TPA ID	: 1015234		

DEPARTMENT OF BIOCHEMISTRY

CHECKMED - IIFB - AHC - PAN INDIA - FY2526

Test Name	Result	Unit	Bio. Ref. Interval	Method
RENAL PROFILE/KIDNEY FUNCTION TEST (RFT/KFT) , *SERUM*				
CREATININE	0.60	mg/dL	0.5-1.04	Creatinine amidohydrolase
.eGFR - ESTIMATED GLOMERULAR FILTRATION RATE	124.04	mL/min/1.73m²	>60	CKD-EPI FORMULA
UREA	17.00	mg/dL	15-36	Urease
BLOOD UREA NITROGEN	**7.9**	mg/dL	8.0 - 23.0	Calculated
URIC ACID	3.50	mg/dL	2.5-6.2	Uricase
CALCIUM	8.80	mg/dL	8.4 - 10.2	Arsenazo-III
PHOSPHORUS, INORGANIC	**4.70**	mg/dL	2.5-4.5	PMA Phenol
SODIUM	141	mmol/L	135-145	Direct ISE
POTASSIUM	4.6	mmol/L	3.5-5.1	ISE (Indirect)
CHLORIDE	106	mmol/L	98 - 107	Direct ISE
PROTEIN, TOTAL	6.90	g/dL	6.4-8.3	Biuret
ALBUMIN	3.70	g/dL	3.5 - 5	Bromocresol Green
GLOBULIN	3.20	g/dL	2.0-3.5	Calculated
A/G RATIO	1.16		0.9-2.0	Calculated

Dr.Yachika
M.B.B.S,M.D(Pathology)
consultant pathologist

SIN No:SR04936357

Patient Name	: Miss.SMITA SHARMA	Collected	: 08/Aug/2025 10:12AM
Age/Gender	: 27 Y 11 M 24 D/F	Received	: 08/Aug/2025 03:49PM
UHID/MR No	: SCHI.0000031585	Reported	: 08/Aug/2025 04:18PM
Visit ID	: SCHIOPV51678	Status	: Final Report
Ref Doctor	: Dr.SELF	Sponsor Name	: CHECKMED PRIVATE LIMITED
Emp/Auth/TPA ID	: 1015234		

DEPARTMENT OF BIOCHEMISTRY

CHECKMED - IIFB - AHC - PAN INDIA - FY2526

Test Name	Result	Unit	Bio. Ref. Interval	Method
HS-CRP (HIGH SENSITIVITY C-REACTIVE PROTEIN) , *SERUM*	0.20	mg/L	<1.0	IMMUNO-TURBIDIMETRY

Comment:

CRP is an acute-phase protein and is useful in the detection and evaluation of infection, tissue injury, and inflammatory disorders. CRP concentrations increase rapidly and dramatically (100-fold or more) in response to tissue injury or inflammation. High-sensitivity CRP (hs-CRP) is more precise than standard CRP and enables a measure of chronic inflammation.Atherosclerosis is an inflammatory disease and hs-CRP has been endorsed by multiple guidelines as a biomarker of atherosclerotic cardiovascular disease risk.

Recommendation by the U.S. Centers for Disease Control and Prevention, and the American Heart Association for assessment of risk for cardiovascular disease in adults-

CARDIOVASCULAR DISEASE RISK	hs-CRP (mg/L)
Low	< 1.0
Average	1.0 – 3.0
High	> 3.0 – 10.0
Possible another source of inflammation/infection	> 10.0

This hs-CRP assay should be used as a means to assess risk of cardiovascular disease or events. A different CRP test (CRP / C-Reactive Protein, Serum) should be used to monitor or assess other inflammatory disorders.

Dr.Tanish Mandal
M.B.B.S,M.D(Pathology)
Consultant Pathologist

SIN No:BI27273999

Patient Name	: Miss.SMITA SHARMA	Collected	: 08/Aug/2025 10:12AM
Age/Gender	: 27 Y 11 M 24 D/F	Received	: 08/Aug/2025 03:19PM
UHID/MR No	: SCHI.0000031585	Reported	: 08/Aug/2025 04:51PM
Visit ID	: SCHIOPV51678	Status	: Final Report
Ref Doctor	: Dr.SELF	Sponsor Name	: CHECKMED PRIVATE LIMITED
Emp/Auth/TPA ID	: 1015234		

DEPARTMENT OF IMMUNOLOGY

CHECKMED - IIFB - AHC - PAN INDIA - FY2526

Test Name	Result	Unit	Bio. Ref. Interval	Method
THYROID PROFILE TOTAL (T3, T4, TSH) , *SERUM*				
TRI-IODOTHYRONINE (T3, TOTAL)	109	ng/dL	84.6-202	ECLIA
THYROXINE (T4, TOTAL)	7.93	µg/dL	5.12-14.06	ECLIA
TSH (Ultrasensitive/4thGen)	2.560	µIU/mL	0.270-4.20	ECLIA

Comment:

For pregnant females	Bio Ref Range for TSH in uIU/ml (As per American Thyroid Association)
First trimester	0.1 - 2.5
Second trimester	0.2 – 3.0
Third trimester	0.3 – 3.0

1. TSH is a glycoprotein hormone secreted by the anterior pituitary. TSH activates production of T3 (Triiodothyronine) & its prohormone T4 (Thyroxine). Increased blood level of T3 and T4 inhibit production of TSH.
2. TSH is elevated in primary hypothyroidism and will be low in primary hyperthyroidism. Elevated or low TSH in the context of normal free thyroxine is often referred to as sub-clinical hypo- or hyperthyroidism respectively.
3. Both T4 & T3 provides limited clinical information as both are highly bound to proteins in circulation and reflects mostly inactive hormone. Only a very small fraction of circulating hormone is free and biologically active. Isolated low T3 is often noticed in elderly
4. Significant variations in TSH can occur with circadian rhythm, hormonal status, stress, sleep deprivation, medication & circulating antibodies.

TSH	T3	T4	FT4	Possible Suggested Conditions correlating with pattern
High	Low	Low	Low	Primary Hypothyroidism, Post Thyroidectomy, Chronic Autoimmune Thyroiditis
High	N	N	N	Subclinical Hypothyroidism, Autoimmune Thyroiditis, Insufficient Hormone Treatment.
N/Low	Low	Low	Low	Secondary and Tertiary Hypothyroidism
Low	High	High	High	Primary Hyperthyroidism, Goitre, Thyroiditis, Drug effects, Early Pregnancy
Low	N	N	N	Subclinical Hyperthyroidism
Low	Low	Low	Low	Central Hypothyroidism, Treatment with Hyperthyroidism
Low	N	High	High	Thyroiditis, Interfering Antibodies
N/Low	High	N	N	T3 Thyrotoxicosis, Non thyroidal causes
High	High	High	High	Pituitary Adenoma; TSHoma/Thyrotropinoma

Dr.Tanish Mandal
M.B.B.S,M.D(Pathology)
Consultant Pathologist

SIN No:SPL25030979

Patient Name	: Miss.SMITA SHARMA	Collected	: 08/Aug/2025 10:12AM
Age/Gender	: 27 Y 11 M 24 D/F	Received	: 08/Aug/2025 01:37PM
UHID/MR No	: SCHI.0000031585	Reported	: 08/Aug/2025 03:52PM
Visit ID	: SCHIOPV51678	Status	: Final Report
Ref Doctor	: Dr.SELF	Sponsor Name	: CHECKMED PRIVATE LIMITED
Emp/Auth/TPA ID	: 1015234		

DEPARTMENT OF CLINICAL PATHOLOGY

CHECKMED - IIFB - AHC - PAN INDIA - FY2526

Test Name	Result	Unit	Bio. Ref. Interval	Method
COMPLETE URINE EXAMINATION (CUE) , *URINE*				
Physical Examination				
COLOUR	PALE YELLOW		PALE YELLOW	Visual
TRANSPARENCY	CLEAR		CLEAR	Physical Measurement
pH	5.50		5-7.5	Double Indicator
SP. GRAVITY	1.020		1.002-1.030	Bromothymol Blue
BIOCHEMICAL EXAMINATION				
URINE PROTEIN	NEGATIVE		NEGATIVE	Protein Error Of Indicator
GLUCOSE	NEGATIVE		NEGATIVE	Glucose Oxidase
URINE BILIRUBIN	NEGATIVE		NEGATIVE	Azo Coupling Reaction
URINE KETONES (RANDOM)	NEGATIVE		NEGATIVE	Sodium Nitro Prusside
UROBILINOGEN	Normal		NORMAL	Modifed Ehrlich Reaction
NITRITE	NEGATIVE		NEGATIVE	Diazotization
LEUCOCYTE ESTERASE	POSITIVE		NEGATIVE	Leucocyte Esterase
CENTRIFUGED SEDIMENT WET MOUNT AND MICROSCOPY				
Pus Cells	8-10	/hpf	0-5	Microscopy
EPITHELIAL CELLS	1-2	/hpf	<10	Microscopy
RBC	ABSENT	/hpf	0-2	Microscopy
CASTS	NIL		0-2 Hyaline Cast	Microscopy
CRYSTALS	ABSENT		ABSENT	Microscopy

Comment:

All urine samples are checked for adequacy and suitability before examination. All abnormal chemical examination are rechecked and verified by manual methods. Microscopy findings are reported as an average of 10 high power fields.

Dr.Yachika
M.B.B.S,M.D(Pathology)
consultant pathologist

SIN No:UR2461000

Patient Name	: Miss.SMITA SHARMA	Collected	: 08/Aug/2025 10:12AM
Age/Gender	: 27 Y 11 M 24 D/F	Received	: 08/Aug/2025 12:31PM
UHID/MR No	: SCHI.0000031585	Reported	: 08/Aug/2025 03:51PM
Visit ID	: SCHIOPV51678	Status	: Final Report
Ref Doctor	: Dr.SELF	Sponsor Name	: CHECKMED PRIVATE LIMITED
Emp/Auth/TPA ID	: 1015234		

DEPARTMENT OF SEROLOGY

CHECKMED - IIFB - AHC - PAN INDIA - FY2526

Test Name	Result	Unit	Bio. Ref. Interval	Method
VDRL (RPR) , *SERUM*	NON REACTIVE		NON-REACTIVE	Slide Flocculation

Comment:

Veneral Disease Research Laboratory (VDRL) test is a non-specific screening test and reactive results must be correlated with supportive clinical, historical and epidemiological evidence to arrive at a final diagnosis. Titers of 1:8 and above are considered significant. False positives may be seen in a variety of acute and chronic conditions. It is advisable to confirm diagnosis by TPHA, FTA-Abs, TPI test.

Biological False Positive Results:

(A) Infectious:

1. Bacterial: Scarlet fever, Pneumococcal pneumonia, Bacterial endocarditis, Leprosy, Tuberculosis.
2. Protozoal: Malaria, Trypanosomiasis.
3. Viral: HIV, Measles, Mumps, Viral hepatitis, Chicken Pox, Infectious mononucleosis, etc.

(B) Non-Infectious:

Pregnancy, Advanced cancer, Multiple blood transfusions, Multiple myeloma, Connective tissue disease.

Biological False Negative Results:

May be due to prozone phenomenon. The test must be repeated with higher dilutions of serum, if clinically indicated.

Comparison of RPR, TPHA tests

Test	Use	Limitations
RPR	Screening test. Follow-up titres for monitoring response to treatment.	Biological false positives possible. Repeat testing should be done with a specific test like TPHAfor low titres such as 1:1, 1:2 and 1:4.
TPHA	More specific than RPR	Less sensitive in early syphilis.

RPR : Rising titres are found in active disease and levels subside after successful treatment.

RPR is a simplified alternative to the conventional VDRL test. It has modified VDRL antigen which contains carbon particles. Due to ease of visualisation, the test is sensitive and the test of choice for serum samples.

***** End Of Report *****

Dr.Yachika
M.B.B.S,M.D(Pathology)
consultant pathologist

SIN No:SR04936357

Patient Name	: Miss.SMITA SHARMA	Collected	: 08/Aug/2025 10:12AM
Age/Gender	: 27 Y 11 M 24 D/F	Received	: 08/Aug/2025 12:31PM
UHID/MR No	: SCHI.0000031585	Reported	: 08/Aug/2025 03:51PM
Visit ID	: SCHIOPV51678	Status	: Final Report
Ref Doctor	: Dr.SELF	Sponsor Name	: CHECKMED PRIVATE LIMITED
Emp/Auth/TPA ID	: 1015234		

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2. This is computer generated medical diagnostics report that has been validated by an Authorized Medical Practitioner/Doctor. The report does not need physical signature.
3. Partial reproduction of this report is not valid and should not be resorted to draw any conclusion.
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5. Results delays may occur due to unforeseen circumstances such as non-availability of kits, equipment breakdown, natural calamities, IT downtime, logistic delays or any other unavoidable event. For certain tests based on analyte stability, criticality of results and in the interest of patient for having appropriate medical diagnosis, the same test may be outsourced to other accredited laboratory.
6. It is presumed that the tests performed are, on the specimen / sample being to the patient named or identified and the verifications of particulars have been confirmed by the patient or his / her representative at the point of generation of said specimen
7. The reported results are restricted to the given specimen only. Results may vary from lab to lab and from time to time for the same parameter for the same patient (within subject biological variation).
8. The patient details along with their results in certain cases like notifiable diseases and as per local regulatory requirements will be communicated to the assigned regulatory bodies
9. The patient samples can be used as part of internal quality control, test verification, data analysis purposes within the testing scope of the laboratory.
10. This report is not valid for medico legal purposes. It is performed to facilitate medical diagnosis only

Dr.Yachika
M.B.B.S,M.D(Pathology)
consultant pathologist

SIN No:SR04936357

Patient Name	: Miss.SMITA SHARMA	Collected	: 08/Aug/2025 10:12AM
Age/Gender	: 27 Y 11 M 24 D/F	Received	: 08/Aug/2025 11:41AM
UHID/MR No	: SCHI.0000031585	Reported	: 08/Aug/2025 01:09PM
Visit ID	: SCHIOPV51678	Status	: Final Report
Ref Doctor	: Dr.SELF	Sponsor Name	: CHECKMED PRIVATE LIMITED
Emp/Auth/TPA ID	: 1015234		

DEPARTMENT OF HAEMATOLOGY

CHECKMED - IIFB - AHC - PAN INDIA - FY2526

Test Name	Result	Unit	Bio. Ref. Interval	Method
COMPLETE BLOOD COUNT (CBC) , *WHOLE BLOOD EDTA*				
HAEMOGLOBIN	**11.8**	g/dL	12-15	Spectrophotometer
PCV	**35.20**	%	36-46	Electronic pulse & Calculation
RBC COUNT	3.9	Million/cu.mm	3.8-4.8	Electrical Impedence
MCV	90.1	fL	83-101	Calculated
MCH	30.3	pg	27-32	Calculated
MCHC	33.7	g/dL	31.5-34.5	Calculated
R.D.W	13	%	11.6-14	Calculated
TOTAL LEUCOCYTE COUNT (TLC)	6,550	cells/cu.mm	4000-10000	Electrical Impedance
DIFFERENTIAL LEUCOCYTIC COUNT (DLC)				
NEUTROPHILS	72.3	%	40-80	Flow cytometry
LYMPHOCYTES	21.5	%	20-40	Flow cytometry
EOSINOPHILS	**0.7**	%	1-6	Flow cytometry
MONOCYTES	5.4	%	2-10	Flow cytometry
BASOPHILS	0.1	%	0-2	Flow cytometry
CORRECTED TLC	6,550	Cells/cu.mm		Calculated
ABSOLUTE LEUCOCYTE COUNT				
NEUTROPHILS	4735.65	Cells/cu.mm	2000-7000	Calculated
LYMPHOCYTES	1408.25	Cells/cu.mm	1000-3000	Calculated
EOSINOPHILS	45.85	Cells/cu.mm	20-500	Calculated
MONOCYTES	353.7	Cells/cu.mm	200-1000	Calculated
BASOPHILS	6.55	Cells/cu.mm	0-100	Calculated
Neutrophil lymphocyte ratio (NLR)	3.36		0.78- 3.53	Calculated
PLATELET COUNT	263000	cells/cu.mm	150000-410000	Electrical impedence
MPV	**7.2**	Fl	8.1-13.9	Calculated

Dr.Yachika
M.B.B.S,M.D(Pathology)
consultant pathologist

SIN No:BED250069885

Patient Name	: Miss.SMITA SHARMA	Collected	: 08/Aug/2025 10:12AM
Age/Gender	: 27 Y 11 M 24 D/F	Received	: 08/Aug/2025 11:42AM
UHID/MR No	: SCHI.0000031585	Reported	: 08/Aug/2025 01:12PM
Visit ID	: SCHIOPV51678	Status	: Final Report
Ref Doctor	: Dr.SELF	Sponsor Name	: CHECKMED PRIVATE LIMITED
Emp/Auth/TPA ID	: 1015234		

DEPARTMENT OF BIOCHEMISTRY

CHECKMED - IIFB - AHC - PAN INDIA - FY2526

Test Name	Result	Unit	Bio. Ref. Interval	Method
GLUCOSE, FASTING , *NAF PLASMA*	86	mg/dL	70-100	GOD - POD

Comment:

As per American Diabetes Guidelines, 2023

Fasting Glucose Values in mg/dL	Interpretation
70-100 mg/dL	Normal
100-125 mg/dL	Prediabetes
≥126 mg/dL	Diabetes
<70 mg/dL	Hypoglycemia

Note:

1.The diagnosis of Diabetes requires a fasting plasma glucose of > or = 126 mg/dL and/or a random / 2 hr post glucose value of > or = 200 mg/dL on at least 2 occasions.

2. Very high glucose levels (>450 mg/dL in adults) may result in Diabetic Ketoacidosis & is considered critical.

Dr.Yachika
M.B.B.S,M.D(Pathology)
consultant pathologist

SIN No:PLF02234739

Patient Name	: Miss.SMITA SHARMA	Collected	: 08/Aug/2025 10:12AM
Age/Gender	: 27 Y 11 M 24 D/F	Received	: 08/Aug/2025 03:43PM
UHID/MR No	: SCHI.0000031585	Reported	: 08/Aug/2025 04:12PM
Visit ID	: SCHIOPV51678	Status	: Final Report
Ref Doctor	: Dr.SELF	Sponsor Name	: CHECKMED PRIVATE LIMITED
Emp/Auth/TPA ID	: 1015234		

DEPARTMENT OF BIOCHEMISTRY

CHECKMED - IIFB - AHC - PAN INDIA - FY2526

Test Name	Result	Unit	Bio. Ref. Interval	Method
HBA1C (GLYCATED HEMOGLOBIN) , *WHOLE BLOOD EDTA*				
HBA1C, GLYCATED HEMOGLOBIN	5.4	%		HPLC
ESTIMATED AVERAGE GLUCOSE (eAG)	108	mg/dL		Calculated

Comment:

Reference Range as per American Diabetes Association (ADA) 2023 Guidelines:

REFERENCE GROUP	HBA1C %
NON DIABETIC	<5.7
PREDIABETES	5.7 – 6.4
DIABETES	≥ 6.5
DIABETICS	
EXCELLENT CONTROL	6 – 7
FAIR TO GOOD CONTROL	7 – 8
UNSATISFACTORY CONTROL	8 – 10
POOR CONTROL	>10

Note: Dietary preparation or fasting is not required.

1. HbA1C is recommended by American Diabetes Association for Diagnosing Diabetes and monitoring Glycemic Control by American Diabetes Association guidelines 2023.
2. Trends in HbA1C values is a better indicator of Glycemic control than a single test.
3. Low HbA1C in Non-Diabetic patients are associated with Anemia (Iron Deficiency/Hemolytic), Liver Disorders, Chronic Kidney Disease. Clinical Correlation is advised in interpretation of low Values.
4. Falsely low HbA1c (below 4%) may be observed in patients with clinical conditions that shorten erythrocyte life span or decrease mean erythrocyte age. HbA1c may not accurately reflect glycemic control when clinical conditions that affect erythrocyte survival are present.
5. In cases of Interference of Hemoglobin variants in HbA1C, alternative methods (Fructosamine) estimation is recommended for Glycemic Control
   A: HbF >25%
   B: Homozygous Hemoglobinopathy.
   (Hb Electrophoresis is recommended method for detection of Hemoglobinopathy)

Dr.Tanish Mandal
M.B.B.S,M.D(Pathology)
Consultant Pathologist

SIN No:EDT250015834

Patient Name	: Miss.SMITA SHARMA	Collected	: 08/Aug/2025 10:12AM
Age/Gender	: 27 Y 11 M 24 D/F	Received	: 08/Aug/2025 12:31PM
UHID/MR No	: SCHI.0000031585	Reported	: 08/Aug/2025 01:11PM
Visit ID	: SCHIOPV51678	Status	: Final Report
Ref Doctor	: Dr.SELF	Sponsor Name	: CHECKMED PRIVATE LIMITED
Emp/Auth/TPA ID	: 1015234		

DEPARTMENT OF BIOCHEMISTRY

CHECKMED - IIFB - AHC - PAN INDIA - FY2526

Test Name	Result	Unit	Bio. Ref. Interval	Method
LIPID PROFILE , *SERUM*				
TOTAL CHOLESTEROL	175	mg/dL	<200	CHE/CHO/POD
TRIGLYCERIDES	74	mg/dL	<150	Enzymatic
HDL CHOLESTEROL	49	mg/dL	>40	CHE/CHO/POD
NON-HDL CHOLESTEROL	126	mg/dL	<130	Calculated
LDL CHOLESTEROL	**111.2**	mg/dL	<100	Calculated
VLDL CHOLESTEROL	14.8	mg/dL	<30	Calculated
CHOL / HDL RATIO	3.57		0-4.97	Calculated
ATHEROGENIC INDEX (AIP)	< 0.01		<0.11	Calculated

Comment:

Reference Interval as per National Cholesterol Education Program (NCEP) Adult Treatment Panel III Report.
Below Table as per Lipid Association of India (LAI) (2023) and Cardiological Society of India (CSI) (2024) Guidelines and Consensus Statements for Dyslipidemia Management:

	Low Risk	Moderate Risk	High Risk	Very High Risk	Extremely High Risk
Total Cholesterol	< 200	< 200	200 – 239	≥ 240	≥ 240
Triglycerides	<150	< 150	150 – 199	200 – 499	≥ 500
LDL	< 100	100 – 129	130 – 159 Target for High Risk: < 70	160 – 189 Target for Very High Risk: < 50	≥ 190 Target for Extreme Risk – Category A,B :< 30,Category C: 10 – 15
HDL	≥ 60	≥ 60	M: <40, F: <50	<30	<30
Non-HDL Cholesterol	< 130	130 – 159	160-189 Target for High Risk: < 100	190 – 219 Target for Very High Risk: < 80	≥220 Target for Extreme Risk – Category A,B :< 60, Category C: 40 – 45

Note: **Low risk** – No known risk factor of cardiovascular disease. **Moderate risk** – Any one risk factor, eg; smoking/hypertension/diabetes mellitus etc. **High risk** – Two or more risk factors without any disease manifestation, chronic kidney disease, long-standing diabetes mellitus existing for >10 years, history of heterozygous familial hypercholesterolemia.
Very high risk – clinical evidence of coronary artery disease, long-standing diabetes mellitus existing for >20 years, etc. **Extremely high risk** – recurrent vascular events.
(Category A- CAD with >1 feature of High risk group, Category B- CAD with >1 feature of very high risk group or poly-vascular disease, Category C- Recurrent CAD with other significant risk factors as high lipoprotein a, extensive coronary calcium)

1. Measurements for Lipids (Especially Triglycerides) can show physiological (Dependent on diet,10-12 hrs fasting pre-test condition) & analytical variations.
2. Lipid Association of India (LAI) recommends screening of all adults (>20 yrs) for Atherosclerotic Cardiovascular Disease (ASCVD) risk factors with lipid profile testing. The association recommends testing also to include Apolipoprotein B & Lipoprotein (a) for stratification and defining LDL – C targets.

Dr.Yachika
M.B.B.S,M.D(Pathology)
consultant pathologist

SIN No:SR04936357

Patient Name	: Miss.SMITA SHARMA	Collected	: 08/Aug/2025 10:12AM
Age/Gender	: 27 Y 11 M 24 D/F	Received	: 08/Aug/2025 12:31PM
UHID/MR No	: SCHI.0000031585	Reported	: 08/Aug/2025 01:11PM
Visit ID	: SCHIOPV51678	Status	: Final Report
Ref Doctor	: Dr.SELF	Sponsor Name	: CHECKMED PRIVATE LIMITED
Emp/Auth/TPA ID	: 1015234		

DEPARTMENT OF BIOCHEMISTRY

CHECKMED - IIFB - AHC - PAN INDIA - FY2526

Test Name	Result	Unit	Bio. Ref. Interval	Method
LIVER FUNCTION TEST (LFT) , *SERUM*				
BILIRUBIN, TOTAL	0.30	mg/dL	0-1.2	DIAZO METHOD
BILIRUBIN CONJUGATED (DIRECT)	0.10	mg/dL	0.0-0.3	Calculated
BILIRUBIN (INDIRECT)	0.20	mg/dL	0-1.1	Dual Wavelength
ALANINE AMINOTRANSFERASE (ALT/SGPT)	14	U/L	<35	Visible with P-5-P
ASPARTATE AMINOTRANSFERASE (AST/SGOT)	20.0	U/L	14-36	UV with P-5-P
AST (SGOT) / ALT (SGPT) RATIO (DE RITIS)	**1.4**		<1.15	Calculated
ALKALINE PHOSPHATASE	73.00	U/L	38-126	p-nitrophenyl phosphate
PROTEIN, TOTAL	6.90	g/dL	6.4-8.3	Biuret
ALBUMIN	3.70	g/dL	3.5 - 5	Bromocresol Green
GLOBULIN	3.20	g/dL	2.0-3.5	Calculated
A/G RATIO	1.16		0.9-2.0	Calculated

Comment:

LFT results reflect different aspects of the health of the liver, i.e., hepatocyte integrity (AST & ALT), synthesis and secretion of bile (Bilirubin, ALP), cholestasis (ALP, GGT), protein synthesis (Albumin) Common patterns seen:

1. Hepatocellular Injury: *AST – Elevated levels can be seen. However, it is not specific to liver and can be raised in cardiac and skeletal injuries.*ALT – Elevated levels indicate hepatocellular damage. It is considered to be most specific lab test for hepatocellular injury. Values also correlate well with increasing BMI. Disproportionate increase in AST, ALT compared with ALP. AST: ALT (ratio) – In case of hepatocellular injury AST: ALT > 1In Alcoholic Liver Disease AST: ALT usually >2. This ratio is also seen to be increased in NAFLD, Wilsons's diseases, Cirrhosis, but the increase is usually not >2.Note- If both SGPT and SGOT are within reference range then AST:ALT (De Ritis ratio) does not have any clinical significance.
2. Cholestatic Pattern:*ALP – Disproportionate increase in ALP compared with AST, ALT. ALP elevation also seen in pregnancy, impacted by age and sex.*Bilirubin (Direct) and GGT elevated- helps to establish hepatic origin.
3. Synthetic function impairment:*Albumin- Liver disease reduces albumin levels, Correlation with PT (Prothrombin Time) helps.
4. Associated tests for assessment of liver fibrosis - Fibrosis-4 and APRI Index.

Dr.Yachika
M.B.B.S,M.D(Pathology)
consultant pathologist

SIN No:SR04936357

Patient Name	: Miss.SMITA SHARMA	Collected	: 08/Aug/2025 10:12AM
Age/Gender	: 27 Y 11 M 24 D/F	Received	: 08/Aug/2025 12:31PM
UHID/MR No	: SCHI.0000031585	Reported	: 08/Aug/2025 03:27PM
Visit ID	: SCHIOPV51678	Status	: Final Report
Ref Doctor	: Dr.SELF	Sponsor Name	: CHECKMED PRIVATE LIMITED
Emp/Auth/TPA ID	: 1015234		

DEPARTMENT OF BIOCHEMISTRY

CHECKMED - IIFB - AHC - PAN INDIA - FY2526

Test Name	Result	Unit	Bio. Ref. Interval	Method
RENAL PROFILE/KIDNEY FUNCTION TEST (RFT/KFT) , *SERUM*				
CREATININE	0.60	mg/dL	0.5-1.04	Creatinine amidohydrolase
.eGFR - ESTIMATED GLOMERULAR FILTRATION RATE	124.04	mL/min/1.73m²	>60	CKD-EPI FORMULA
UREA	17.00	mg/dL	15-36	Urease
BLOOD UREA NITROGEN	**7.9**	mg/dL	8.0 - 23.0	Calculated
URIC ACID	3.50	mg/dL	2.5-6.2	Uricase
CALCIUM	8.80	mg/dL	8.4 - 10.2	Arsenazo-III
PHOSPHORUS, INORGANIC	**4.70**	mg/dL	2.5-4.5	PMA Phenol
SODIUM	141	mmol/L	135-145	Direct ISE
POTASSIUM	4.6	mmol/L	3.5-5.1	ISE (Indirect)
CHLORIDE	106	mmol/L	98 - 107	Direct ISE
PROTEIN, TOTAL	6.90	g/dL	6.4-8.3	Biuret
ALBUMIN	3.70	g/dL	3.5 - 5	Bromocresol Green
GLOBULIN	3.20	g/dL	2.0-3.5	Calculated
A/G RATIO	1.16		0.9-2.0	Calculated

Dr.Yachika
M.B.B.S,M.D(Pathology)
consultant pathologist

SIN No:SR04936357

Patient Name	: Miss.SMITA SHARMA	Collected	: 08/Aug/2025 10:12AM
Age/Gender	: 27 Y 11 M 24 D/F	Received	: 08/Aug/2025 03:49PM
UHID/MR No	: SCHI.0000031585	Reported	: 08/Aug/2025 04:18PM
Visit ID	: SCHIOPV51678	Status	: Final Report
Ref Doctor	: Dr.SELF	Sponsor Name	: CHECKMED PRIVATE LIMITED
Emp/Auth/TPA ID	: 1015234		

DEPARTMENT OF BIOCHEMISTRY

CHECKMED - IIFB - AHC - PAN INDIA - FY2526

Test Name	Result	Unit	Bio. Ref. Interval	Method
HS-CRP (HIGH SENSITIVITY C-REACTIVE PROTEIN) , *SERUM*	0.20	mg/L	<1.0	IMMUNO-TURBIDIMETRY

Comment:

CRP is an acute-phase protein and is useful in the detection and evaluation of infection, tissue injury, and inflammatory disorders. CRP concentrations increase rapidly and dramatically (100-fold or more) in response to tissue injury or inflammation. High-sensitivity CRP (hs-CRP) is more precise than standard CRP and enables a measure of chronic inflammation.Atherosclerosis is an inflammatory disease and hs-CRP has been endorsed by multiple guidelines as a biomarker of atherosclerotic cardiovascular disease risk.

Recommendation by the U.S. Centers for Disease Control and Prevention, and the American Heart Association for assessment of risk for cardiovascular disease in adults-

CARDIOVASCULAR DISEASE RISK	hs-CRP (mg/L)
Low	< 1.0
Average	1.0 – 3.0
High	> 3.0 – 10.0
Possible another source of inflammation/infection	> 10.0

This hs-CRP assay should be used as a means to assess risk of cardiovascular disease or events. A different CRP test (CRP / C-Reactive Protein, Serum) should be used to monitor or assess other inflammatory disorders.

Dr.Tanish Mandal
M.B.B.S,M.D(Pathology)
Consultant Pathologist

SIN No:BI27273999

Patient Name	: Miss.SMITA SHARMA	Collected	: 08/Aug/2025 10:12AM
Age/Gender	: 27 Y 11 M 24 D/F	Received	: 08/Aug/2025 03:19PM
UHID/MR No	: SCHI.0000031585	Reported	: 08/Aug/2025 04:51PM
Visit ID	: SCHIOPV51678	Status	: Final Report
Ref Doctor	: Dr.SELF	Sponsor Name	: CHECKMED PRIVATE LIMITED
Emp/Auth/TPA ID	: 1015234		

DEPARTMENT OF IMMUNOLOGY

CHECKMED - IIFB - AHC - PAN INDIA - FY2526

Test Name	Result	Unit	Bio. Ref. Interval	Method
THYROID PROFILE TOTAL (T3, T4, TSH) , *SERUM*				
TRI-IODOTHYRONINE (T3, TOTAL)	109	ng/dL	84.6-202	ECLIA
THYROXINE (T4, TOTAL)	7.93	µg/dL	5.12-14.06	ECLIA
TSH (Ultrasensitive/4thGen)	2.560	µIU/mL	0.270-4.20	ECLIA

Comment:

For pregnant females	Bio Ref Range for TSH in uIU/ml (As per American Thyroid Association)
First trimester	0.1 - 2.5
Second trimester	0.2 – 3.0
Third trimester	0.3 – 3.0

1. TSH is a glycoprotein hormone secreted by the anterior pituitary. TSH activates production of T3 (Triiodothyronine) & its prohormone T4 (Thyroxine). Increased blood level of T3 and T4 inhibit production of TSH.
2. TSH is elevated in primary hypothyroidism and will be low in primary hyperthyroidism. Elevated or low TSH in the context of normal free thyroxine is often referred to as sub-clinical hypo- or hyperthyroidism respectively.
3. Both T4 & T3 provides limited clinical information as both are highly bound to proteins in circulation and reflects mostly inactive hormone. Only a very small fraction of circulating hormone is free and biologically active. Isolated low T3 is often noticed in elderly
4. Significant variations in TSH can occur with circadian rhythm, hormonal status, stress, sleep deprivation, medication & circulating antibodies.

TSH	T3	T4	FT4	Possible Suggested Conditions correlating with pattern
High	Low	Low	Low	Primary Hypothyroidism, Post Thyroidectomy, Chronic Autoimmune Thyroiditis
High	N	N	N	Subclinical Hypothyroidism, Autoimmune Thyroiditis, Insufficient Hormone Treatment.
N/Low	Low	Low	Low	Secondary and Tertiary Hypothyroidism
Low	High	High	High	Primary Hyperthyroidism, Goitre, Thyroiditis, Drug effects, Early Pregnancy
Low	N	N	N	Subclinical Hyperthyroidism
Low	Low	Low	Low	Central Hypothyroidism, Treatment with Hyperthyroidism
Low	N	High	High	Thyroiditis, Interfering Antibodies
N/Low	High	N	N	T3 Thyrotoxicosis, Non thyroidal causes
High	High	High	High	Pituitary Adenoma; TSHoma/Thyrotropinoma

Dr.Tanish Mandal
M.B.B.S,M.D(Pathology)
Consultant Pathologist

SIN No:SPL25030979

Patient Name	: Miss.SMITA SHARMA	Collected	: 08/Aug/2025 10:12AM
Age/Gender	: 27 Y 11 M 24 D/F	Received	: 08/Aug/2025 01:37PM
UHID/MR No	: SCHI.0000031585	Reported	: 08/Aug/2025 03:52PM
Visit ID	: SCHIOPV51678	Status	: Final Report
Ref Doctor	: Dr.SELF	Sponsor Name	: CHECKMED PRIVATE LIMITED
Emp/Auth/TPA ID	: 1015234		

DEPARTMENT OF CLINICAL PATHOLOGY

CHECKMED - IIFB - AHC - PAN INDIA - FY2526

Test Name	Result	Unit	Bio. Ref. Interval	Method
COMPLETE URINE EXAMINATION (CUE) , *URINE*				
Physical Examination				
COLOUR	PALE YELLOW		PALE YELLOW	Visual
TRANSPARENCY	CLEAR		CLEAR	Physical Measurement
pH	5.50		5-7.5	Double Indicator
SP. GRAVITY	1.020		1.002-1.030	Bromothymol Blue
BIOCHEMICAL EXAMINATION				
URINE PROTEIN	NEGATIVE		NEGATIVE	Protein Error Of Indicator
GLUCOSE	NEGATIVE		NEGATIVE	Glucose Oxidase
URINE BILIRUBIN	NEGATIVE		NEGATIVE	Azo Coupling Reaction
URINE KETONES (RANDOM)	NEGATIVE		NEGATIVE	Sodium Nitro Prusside
UROBILINOGEN	Normal		NORMAL	Modifed Ehrlich Reaction
NITRITE	NEGATIVE		NEGATIVE	Diazotization
LEUCOCYTE ESTERASE	POSITIVE		NEGATIVE	Leucocyte Esterase
CENTRIFUGED SEDIMENT WET MOUNT AND MICROSCOPY				
Pus Cells	8-10	/hpf	0-5	Microscopy
EPITHELIAL CELLS	1-2	/hpf	<10	Microscopy
RBC	ABSENT	/hpf	0-2	Microscopy
CASTS	NIL		0-2 Hyaline Cast	Microscopy
CRYSTALS	ABSENT		ABSENT	Microscopy

Comment:

All urine samples are checked for adequacy and suitability before examination. All abnormal chemical examination are rechecked and verified by manual methods. Microscopy findings are reported as an average of 10 high power fields.

Dr.Yachika
M.B.B.S,M.D(Pathology)
consultant pathologist

SIN No:UR2461000

Patient Name	: Miss.SMITA SHARMA	Collected	: 08/Aug/2025 10:12AM
Age/Gender	: 27 Y 11 M 24 D/F	Received	: 08/Aug/2025 12:31PM
UHID/MR No	: SCHI.0000031585	Reported	: 08/Aug/2025 03:51PM
Visit ID	: SCHIOPV51678	Status	: Final Report
Ref Doctor	: Dr.SELF	Sponsor Name	: CHECKMED PRIVATE LIMITED
Emp/Auth/TPA ID	: 1015234		

DEPARTMENT OF SEROLOGY

CHECKMED - IIFB - AHC - PAN INDIA - FY2526

Test Name	Result	Unit	Bio. Ref. Interval	Method
VDRL (RPR) , *SERUM*	NON REACTIVE		NON-REACTIVE	Slide Flocculation

Comment:

Veneral Disease Research Laboratory (VDRL) test is a non-specific screening test and reactive results must be correlated with supportive clinical, historical and epidemiological evidence to arrive at a final diagnosis. Titers of 1:8 and above are considered significant. False positives may be seen in a variety of acute and chronic conditions. It is advisable to confirm diagnosis by TPHA, FTA-Abs, TPI test.

Biological False Positive Results:

(A) Infectious:

1. Bacterial: Scarlet fever, Pneumococcal pneumonia, Bacterial endocarditis, Leprosy, Tuberculosis.
2. Protozoal: Malaria, Trypanosomiasis.
3. Viral: HIV, Measles, Mumps, Viral hepatitis, Chicken Pox, Infectious mononucleosis, etc.

(B) Non-Infectious:

Pregnancy, Advanced cancer, Multiple blood transfusions, Multiple myeloma, Connective tissue disease.

Biological False Negative Results:

May be due to prozone phenomenon. The test must be repeated with higher dilutions of serum, if clinically indicated.

Comparison of RPR, TPHA tests

Test	Use	Limitations
RPR	Screening test. Follow-up titres for monitoring response to treatment.	Biological false positives possible. Repeat testing should be done with a specific test like TPHAfor low titres such as 1:1, 1:2 and 1:4.
TPHA	More specific than RPR	Less sensitive in early syphilis.

RPR : Rising titres are found in active disease and levels subside after successful treatment.

RPR is a simplified alternative to the conventional VDRL test. It has modified VDRL antigen which contains carbon particles. Due to ease of visualisation, the test is sensitive and the test of choice for serum samples.

***** End Of Report *****

Dr.Yachika
M.B.B.S,M.D(Pathology)
consultant pathologist

SIN No:SR04936357

Patient Name	: Miss.SMITA SHARMA	Collected	: 08/Aug/2025 10:12AM
Age/Gender	: 27 Y 11 M 24 D/F	Received	: 08/Aug/2025 12:31PM
UHID/MR No	: SCHI.0000031585	Reported	: 08/Aug/2025 03:51PM
Visit ID	: SCHIOPV51678	Status	: Final Report
Ref Doctor	: Dr.SELF	Sponsor Name	: CHECKMED PRIVATE LIMITED
Emp/Auth/TPA ID	: 1015234		

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5. Results delays may occur due to unforeseen circumstances such as non-availability of kits, equipment breakdown, natural calamities, IT downtime, logistic delays or any other unavoidable event. For certain tests based on analyte stability, criticality of results and in the interest of patient for having appropriate medical diagnosis, the same test may be outsourced to other accredited laboratory.
6. It is presumed that the tests performed are, on the specimen / sample being to the patient named or identified and the verifications of particulars have been confirmed by the patient or his / her representative at the point of generation of said specimen
7. The reported results are restricted to the given specimen only. Results may vary from lab to lab and from time to time for the same parameter for the same patient (within subject biological variation).
8. The patient details along with their results in certain cases like notifiable diseases and as per local regulatory requirements will be communicated to the assigned regulatory bodies
9. The patient samples can be used as part of internal quality control, test verification, data analysis purposes within the testing scope of the laboratory.
10. This report is not valid for medico legal purposes. It is performed to facilitate medical diagnosis only

Dr.Yachika
M.B.B.S,M.D(Pathology)
consultant pathologist

SIN No:SR04936357

Patient Name	: Miss. SMITA SHARMA	Age	: 27 Y/F
UHID	: SCHI.0000031585	OP Visit No	: SCHIOPV51678
Conducted By:	: Dr. MUKESH K GUPTA	Conducted Date	: 08-08-2025 15:22
Referred By	: SELF		

Protocol	:	Bruce Protocol
Medication	:	
Target Heart Rate	:	193 BPM
Heart Rate Achieved	:	193 BPM
Percentage of THR Achieved	:	100%
Maximum Blood Pressure	:	138/90 mmHg
Total Exercise Duration	:	09:34 Min.
Maximum Worked Attained	:	11.90 Mets
Reason for termination	:	THR Achieved.

Comments

- Basal ECG NSR.
- Appropriate HR response.
- Appropriate BP response.
- No significant changes with standing and hyperventilation.
- Good exercise tolerance.
- No significant ST segment depression over baseline during exercise or recovery period.
- No crepts or rhonchi.
- Arrhythmia none.
- Chest pain absent.

Summary

- Test is negative for provocable myocardial ischemia.
- Good exercise tolerance.
- Appropriate BP response.

Please correlate clinically
Not valid for medico legal purpose.

Dr. M K Gupta
M.B.B.S, MD,FIACM
Senior Consultant Cardiologist

Patient Name	: Miss. SMITA SHARMA	Age	: 27 Y/F
UHID	: SCHI.0000031585	OP Visit No	: SCHIOPV51678
Conducted By:	: Dr. MUKESH K GUPTA	Conducted Date	: 08-08-2025 15:22
Referred By	: SELF		

Patient Name	: Miss. SMITA SHARMA	Age	: 27 Y/F
UHID	: SCHI.0000031585	OP Visit No	: SCHIOPV51678
Conducted By:	:	Conducted Date	:
Referred By	: SELF		

Patient Name	: Miss. SMITA SHARMA	Age	: 27 Y/F
UHID	: SCHI.0000031585	OP Visit No	: SCHIOPV51678
Conducted By	:	Conducted Date	:
Referred By	: SELF		